Anderson Diagnostic Services Private Limited
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Peripheral Blood Karyotyping

Patient name	: **Mrs.Karpagavidhya (UMR2601043940)**	**PIN**	: **CBC0000002136**
Gender/ Age	: **Female / 37 Years**	**Sample Number**	: **260161295**
Specimen	: **Peripheral blood**	**Sample collection date**	: **22-02-2026**
Referring Clinician	: **Dr. Aruna**	**Sample receipt date**	: **23-02-2026**
Hospital/Clinic	: **A4 Fertility Centre - (A Unit Of A4 Hospital)-Virugambakkam**	**Report Date**	: **17-03-2026**

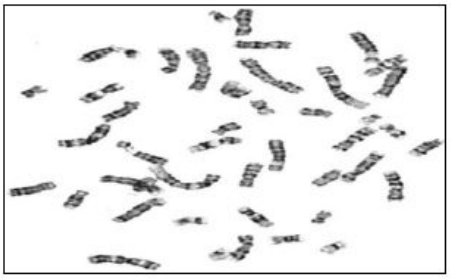
MC-6558

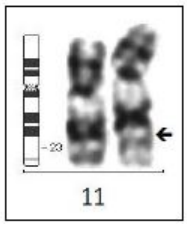
Test Indication

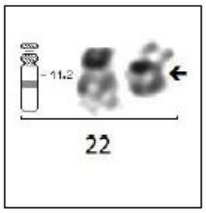
To rule out gross chromosomal abnormality

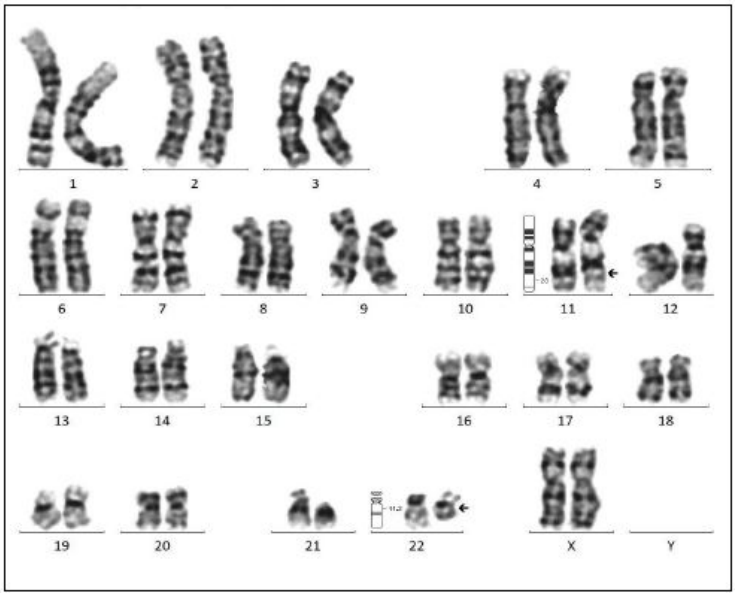
Result

International System for Human Cytogenomic Nomenclature (ISCN 2024): 46,XX,t(11;22)(q23;q11.2)









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PGT-A, PGT-M & PGT-SR | Clinical Exome Sequencing | Microarray | Male Infertility | Recurrent Pregnancy Loss | Carrier Screening | Amniotic Fluid Testing | POC Analysis | NIPS
Fertility Genetics | Genetic Counseling | Oncogenetics | Neurogenetics | Infectious Genetics | New Born Screening

Metaphase analysed : 25	Autosome : Abnormal
Estimated band resolution : 475	Sex chromosome : Normal

Interpretation

Karyotype shows a female with an apparently balanced translocation involving chromosome 11 and 22 with the breakpoints 11q23 and 22q11.2

Comments

Usually this arrangement has no effect on development or general health because no genes have been lost or gained. However, the carrier of a balanced translocation has an increased risk of infertility, recurrent abortions and live-born offspring with chromosome imbalances like Emanuel syndrome.

Recommendations

Genetic counseling advised (to include review of partners' karyotype). Prenatal diagnosis of all subsequent pregnancies is strongly recommended.

Test Methodology

A 72-hour PHA-M stimulated culture of the received peripheral blood sample was processed, according to a protocol adapted from the AGT Cytogenetics Laboratory Manual, Third Edition. Numerical and structural chromosomal abnormalities were ruled out at a banding resolution suitable for the referral indication, in accordance with current ISCN guidelines.

Limitations

- All genetic disorders cannot be ruled out by conventional karyotyping.
- The accuracy of the test is about 99%.
- G-banded analysis cannot detect small rearrangements and submicroscopic deletions.
- Low-level mosaicism may not be detected.

References

1. The AGT Cytogenetics Laboratory Manual Third Edition (1997) *Editors: Margaret J. Barch, TuridKnutsen, Jack L. Spurbeck.*
2. ISCN- An International System for Human Cytogenetic Nomenclature (2024) *Editors: Ros J. Hastings, Sarah Moore, Nicole Chia.*

This report has been reviewed and approved by:

Dr. R. Deepika., Ph.D.,	**Dr. Teena Koshy.**, Ph.D.,	**Dr. Suriya kumar G**
Consultant Cytogeneticist	Consultant Cytogeneticist	Director
Anderson Diagnostics & Labs	Anderson Diagnostics & Labs	Anderson Diagnostics & Labs